

DRAFT FOR MEDICAL REVIEW — NOT PUBLISHED

This article is written but **not live**, and is not linked from anywhere on the site. It cannot publish until a reviewing clinician is named: the build refuses an article marked `clinical` with no named reviewer, which is why this page exists rather than a live URL. Please read it as the page a donor will see, mark anything that is wrong or that you would word differently, and complete the sign-off on the last page.

How often can you donate blood

The minimum interval between whole blood donations, why it differs for men and women, and why platelet donation follows a completely different clock.

Intended URL	https://raktify.choudhari.ngo/learn/how-often-can-you-donate-blood
Written by	Choudhari EduHealth India Foundation
Status	draft (clinical)
Reviewer	none named — this is what is being requested

Specific questions for the reviewer

- Please confirm every interval printed here, including the different figures for men and women, against your own practice as well as against the NBTC standard.
- The section *Platelets follow a different clock entirely* prints intervals and an annual limit for apheresis platelets. These are the figures we are least confident about for a public page - please check them closely.
- *What the interval is counted from* states what the clock runs from. Confirm that is right in all cases, including a donation that was started and abandoned.
- Is there anything here a donor could read as encouragement to donate more often than is safe?

The article, as it will publish

A whole blood donation takes about 450 mL — roughly a tenth of the blood in an adult body. The fluid is replaced within a day or two. The red cells are not: the body has to manufacture them again, and that needs iron, which comes back more slowly than anything else involved.

The interval between donations exists for that reason and no other. It is not a limit on how much you are allowed to help. It is the time it takes for one donation to stop costing you anything.

Whole blood

Men: 90 days. Women: 120 days.

The difference is about iron reserves rather than about anything else, and it is why a blood bank will ask. Women of donating age generally hold lower iron stores and lose iron monthly, so the same donation takes longer to make good.

Those are minimums. There is nothing wrong with leaving longer, and a donor who comes every six months for twenty years gives far more blood than one who over-commits and stops.

Plasma and cryoprecipitate are made from the same whole blood collection, so donating for them follows the same clock.

Platelets follow a different clock entirely

Platelets are collected in two ways, and the interval depends on which.

What you donate	Minimum gap
Whole blood, or plasma and cryoprecipitate made from it	90 days (men) / 120 days (women)
Random donor platelets, separated from a whole blood unit	14 days
Single donor platelets, collected by apheresis	2 days

Apheresis is the striking one. The machine takes blood from one arm, separates out the platelets and returns the red cells to you, so almost no iron leaves your body — which is why the gap can be measured in days rather than months. Your platelet count recovers within about two days.

That matters because platelets expire in five days from collection. There is no way to stockpile them, so a patient on chemotherapy or a dengue ward needing platelets this week depends on someone donating this week. Apheresis donors who can come back repeatedly are, for that reason, disproportionately valuable.

Apheresis takes longer in the chair — closer to an hour than ten minutes — and not every blood bank runs it.

Why a shelf life makes the interval matter

A collected unit does not wait indefinitely for the patient who needs it.

Component	Keeps for
Red cells and whole blood	about 35 days
Platelets	5 days
Plasma and cryoprecipitate	about a year

So the demand on donors is continuous rather than seasonal. A blood bank that filled its refrigerator in April has nothing left from April by June. This is the real reason the appeals never stop, and the reason a steady donor who returns on schedule is worth more than a one-time drive.

What the interval is counted from

The date of your last donation, not the date you registered, not the date of the camp you attended, and not the date a result came back. Raktify records the donation when the blood bank enters it, and the next date you become eligible is calculated from that — gender-aware, enforced in the database rather than in a form, so an interval cannot be waived by a mistake in an app.

A few things do not restart the clock:

- **Being deferred on the day without donating.** Nothing was collected, so nothing has to be replaced. You can come back as soon as the deferral that stopped you has passed.
- **A unit that is later discarded.** If a test result means the unit cannot be used, your body still gave the blood, and the recovery time is unchanged.
- **Registering on a platform, or responding to an alert.** Only a recorded donation counts.

A separate temporary deferral can extend the wait beyond the normal interval — a vaccination, surgery or illness in the meantime starts its own clock, and whichever date is later is the one that applies.

What Raktify does with the date

Once your eligibility date is in the future, the platform stops sending you donor alerts until it passes. That is deliberate and it is the main reason the alerts are worth reading: a message from Raktify means a patient near you needs your group **and** you are someone who can actually help today.

You will also see your own next-eligible date on your donor dashboard, which saves the arithmetic.

Donating more often than this is not a kindness

The interval is not a bureaucratic ceiling to be negotiated down in an emergency. A donor whose iron has not recovered risks being anaemic themselves, and the unit they give is not better for having come early.

The way to help more often is to bring someone with you. One donor who recruits three others contributes four units in the time they could have given one.

Sources

- National Blood Transfusion Council (NBTC), Standards for Blood Banks and Blood Transfusion Services
- Raktify blood component reference data, confirmed by the foundation's medical advisor on 10 July 2026

Reviewer sign-off

Raktify records the reviewing clinician by name on the published page, with the review date. Nothing below is optional — an unnamed review cannot be recorded.

Full name

Qualification (e.g. MD Pathology / MD Transfusion Medicine)

Medical council registration number

Verdict

- ☐ Approved as written.
- ☐ Approved with the amendments I have marked on this document.
- ☐ Not approved — see my notes.

Review date

Next review due (we default to 12 months unless you say otherwise)

Signature

Returned sign-offs are filed in docs/medical-review/ alongside the reference-data review of 10 July 2026, and the reviewer name is published on the article itself.